Supplemental Research Memo: CASOMB-Certified Sex Offender Treatment Providers, Treatment-Completion Policy, and Litigation Support for *Newanforbi v. Palacios et al.*

TL;DR

- California's 2024–2025 regulatory reforms (15 CCR § 3574, effective April 17, 2025) and the ACSOL settlement that produced them fundamentally reshaped when a provider must find treatment "complete" — creating a documented, enforceable "good cause" standard for continued retention that a provider who kept a client in treatment past clinical necessity would violate. This is the single strongest doctrinal anchor for the fraud/negligence/UCL theories.
- HOPE Psychotherapy, Inc. (d/b/a HOPE Program) is a confirmed CDCR-contracted, CASOMB-certified provider that self-describes as "California's largest sex offender rehabilitation initiative"; CPC (Counseling & Psychotherapy Center, Inc.) is a comparable multistate provider whose public materials illuminate industry compensation and completion practices.
- CASOMB's own 2024–2025 complaint data shows the single largest category of complaints concerned providers not crediting prior treatment gains and clients "not having a clear understanding of their pathway to treatment completion" — the exact conduct at issue — but CASOMB's enforcement powers are limited to admonition, probation, and decertification, making civil litigation the primary vehicle for damages.

Key Findings

1. Treatment completion & discharge is now heavily regulated — and provider discretion is bounded by "good cause"

- Under **15 CCR § 3574** (adopted as an emergency regulation 10/14/2024; Certificate of Compliance filed and effective **4/17/2025**, Register 2025 No. 16), the sex offender management program has three phases (Curriculum/active, Maintenance, Aftercare) plus Containment Team Meetings (CTMs) and polygraphs.

- **§ 3574(b)**: participation must be "no less than one year, up to the entire period of parole." ca cornell
- **§ 3574(c)**: "The sex offender treatment provider prescribes the number of treatment hours per week as determined by the supervised person's risks, needs, responsivity, and offense-free time in the community."
- **§ 3574(e)(2)**: the second CTM (the "one-year CTM," within nine months of the first) is "the first review to determine whether the supervised person's sex offender management program requirements have been satisfied, completing their statutory requirement to successfully complete a sex offender management program."
- **§ 3574(f)** — the critical provision: "When the Containment Team makes a determination to retain a participant in a sex offender management program, the determination shall be supported by good cause, as defined in Section 3000. A determination to retain a supervised person … shall be stated in writing, signed by the sex offender treatment provider and parole agent on the CDCR Form 3043 … [and] delivered to the supervised person within 30 calendar days of the CTM." The CDCR rulemaking record (NCR 25-01) confirms verbatim that "A Level of Service or Case Management Inventory (LS/CMI) score of 11 or higher and a Stable-2007 score of four or higher (for males only) were established as reasons to retain a participant"; the additional enumerated good-cause reasons are a current polygraph with deception found or new disclosures of technical/law violations, or a parole violation within one year related to the sex offense. The record further notes the Stable-2007 "cannot be utilized as criteria for retention" for females "due to the tool not being valid for females at this time."
- **Litigation significance**: After one year, retention is the exception requiring written, signed, criteria-based justification on Form 3043. A provider who kept a client in treatment without documented good cause, or who fabricated/overstated a good-cause basis, acted **contrary to a specific regulatory command** — highly probative of negligence per se and of the "unlawful" prong of the UCL.

2. The regulation is the product of an ACSOL settlement premised on exactly this abuse

- The § 3574 framework exists because the Alliance for Constitutional Sex Offense Laws (ACSOL) sued CDCR in **Los Angeles Superior Court (July 2023)** challenging an unwritten CDCR policy that required all registrants to remain in treatment the entire parole term. CDCR settled in writing in **May 2024**; per ACSOL, Executive Director Janice Bellucci stated: "This agreement is a significant victory for almost 7,000 registrants currently on parole … No longer will every registrant on parole be required to undergo treatment the entire time they are on parole." The settlement required annual completion reviews and codification of PC 3008(d) into Title 15.

- The 2023 complaint alleged some plaintiffs "have been required to continue treatment even after being told by a treatment provider that they completed a treatment program," and Bellucci framed the mechanism: "The fact that a registrant is required to undergo treatment is often used to keep a registrant on parole … they are told that they cannot be released from parole because they are still undergoing treatment."

- A **San Mateo County Superior Court** judge (March 2023) ordered termination of treatment for a registrant who had completed a three-year program but was told to restart. Per ACSOL, that ruling followed a hearing "in which a CA Department of Corrections (CDCR) representative testified that 'a sex offender cannot successfully "complete" a sex offender management treatment program' while on parole" — direct evidence of an industry/agency mindset conflicting with PC 3008(d)(2), which authorizes courts to disapprove further program participation.

- The financial-motive theory is echoed even in ACSOL's public comment threads: one published comment (May 2024) states, "I am weary about these for-profit treatment programs willingness to basically, 'Give up the business = $.'"

3. CASOMB completion standards (2018 Guidelines) — structured clinical judgment, discharge planning from intake

- "Successful treatment completion" is defined as having "demonstrated sufficient progress in meeting the goals and objectives of an individualized treatment plan." Casomb casomb On completion the client "shall receive a statement of successful treatment completion," Casomb casomb a copy of which is submitted to CASOMB and to the supervising agency.

- The Guidelines direct structured clinical judgment (not a scored instrument), and — critically for a fraud/negligence theory — state verbatim (2018 Guidelines, p. 5, item 15): "Discharge planning starts at intake. The final determination regarding whether an individual has 'completed' treatment should never come as a surprise to the client. Regular reviews of progress in which the treatment provider and client collaborate about the nature, goals, and objectives of treatment along with the criteria for assessing progress and completion are necessary."

- The Guidelines note treatment length should track risk/criminogenic need (RNR), so higher-risk clients receive longer, more intense treatment — a provider's legitimate defense — but they also recognize obstacles "not caused by or under the control of the individual" casomb and warn that some clients discontinue before completion "because they cannot afford the cost of treatment." casomb

- CASOMB "has the authority to conduct audits of certified provider agencies. One focus of such audits will be the provider agency's decision process for determining treatment

completion and the provider agency's outcomes using that process."

4. The 1502/3043 forms and the parole–provider interface

- The operative form for a retain-vs-complete decision is **CDCR Form 3043** (Containment Team Meeting / Earned Discharge Consideration Committee), signed by both the treatment provider and the parole agent, and delivered to the client within 30 days.
- The **CDCR 1502 Activity Report** is used by the parole agent/unit supervisor to *re-refer* a completed client back into treatment if behavior changes raise re-offense risk (§ 3574(g)). On re-referral the provider must re-run dynamic and violence risk assessments; if no increased risk is found, "the re-referral shall be considered closed, and the sex offender management program requirements remain completed." cornell
- The provider's clinical recommendation is central: the provider "prescribes the number of treatment hours per week" cornell (§ 3574(c)) and co-signs the retain/complete determination on Form 3043. This gives the provider substantial control over the client's parole trajectory — establishing the causal link between provider conduct and the parole-supervision harm alleged.

5. Regulatory complaints & enforcement against CASOMB providers

- **CASOMB 2025 Year-End Report** (verbatim): "The Complaints subcommittee received twenty-six complaints in 2025; however, 5 did not fall under CASOMB's purview for processing and investigation." Separately, **18 complaints carried over from 2024**, of which **13 "revolved around agencies ensuring that previous treatment gets considered when receiving a new or transferred client into their agency"** — including allegations that clients formerly in the maintenance phase "were removed from maintenance, simply due to the transfer between agencies." casomb
- 2025 investigated allegations included: "clients not having a clear understanding of their pathway to treatment completion," clients "not being involved in the creation and maintenance of their treatment plan," "making changes to treatment dosage without justification based on current changes to risk-relevant factors," casomb failure to sign treatment plans, and boundary/ethics violations (including a sexual relationship with a client).
- **2025 sanctions**: "decertification of individual providers, and one agency and one provider received letters of admonishment." casomb CASOMB's sanction ladder is: letter of admonition, mutual agreement, probation, or decertification. Casomb casomb As of 12/31/2025, CASOMB had a total of **69 certified treatment provider agencies** (411 total individual certified providers).

- Process: the complaint must "allege a specific violation of the Provider Agency or Treatment Provider Certification Requirements"; Casomb ethics/certification violations go to the Complaints Committee, then the Certification Committee, which may "direct CASOMB staff to notify the appropriate licensing board" (e.g., the Board of Behavioral Sciences). Critically, CASOMB has **no power to award damages** — it is a certification body — so civil litigation is the only avenue for compensatory or punitive relief.

- I found **no public record of a specific CASOMB complaint, BBS/Board of Psychology disciplinary action, or prior lawsuit naming HOPE Psychotherapy, Katie Palacios, Drew Kingston, or CPC** by name (CASOMB's published data is anonymized and does not identify agencies). This should be confirmed via direct CASOMB and licensing-board records requests.

6. Financial structure & incentive analysis

- **CPC** publicly states its programs "are funded by government contracts, Medicaid, private insurance, and self-pay," Cpcamerica and that in its outpatient groups "the clients pay for their own treatment at a standard rate." CPC "operate[s] specialized sex offender management and treatment programs in 80+ locations throughout 11 states."

- **CDCR/DAPO** contracts for treatment through the Division of Rehabilitative Programs (DRP); PC 3007 references "any sex offender treatment contract funded by the Department." CA During the mandatory Curriculum/Maintenance phases treatment is state-funded; the **Aftercare phase is self-pay** ("Persons voluntarily participating in the aftercare phase are responsible for any costs associated with participation," ca Legal Information Institute § 3574(a)(3)(A)).

- CDCR service contracts of this type are priced via an "Exhibit B-1 Rate Sheet," Governmentnavigator implying per-unit billing (per group session, individual session, assessment, polygraph), consistent with the documented dosage minimums (one individual session/month; one group session/week, two if designated HRSO). **The exact contract number C5613216-D, its dollar value, term, and per-unit rates could not be located in public sources** and would require a California Public Records Act (CPRA) request to CDCR's Office of Business Services / Contracts Management Branch (m_cdcrobscontracts@cdcr.ca.gov), a Cal eProcure contract-award search, or an Open FI$Cal (open.fiscal.ca.gov) vendor-payment query.

- **The incentive theory is structurally sound**: if a provider is paid per session or per client-month, every additional month a client is retained past clinical necessity generates additional revenue. Combined with (a) CASOMB's finding that the top complaint category is failure to credit prior treatment, and (b) CDCR's own testimony that completion is treated as practically unattainable, the financial conflict of interest is more than speculative — though the per-unit rate should be obtained to quantify it.

7. HOPE vs. CPC — comparative profile

- **HOPE Psychotherapy, Inc. (d/b/a HOPE Program)**: Northern California–headquartered (Hayward), "Founded in September 2008 by Shannon Smith, LCSW" (per HOPE's own website), who is listed as Chief Executive Officer. CASOMB-certified; **16 locations** across California (including Stockton, Hayward, San Francisco, San Diego, Santa Maria, Salinas, Capitola, and San Mateo). Self-describes as "California's largest sex offender rehabilitation initiative." HOPE Program **Confirmed as a CDCR SOMP contracted provider** (listed among providers in CDCR's FY2019-20 Sex Offender Treatment Program Outcome Evaluation Report). Methodology: CBT, Good Lives Model, RNR, with Static-99R / STABLE-2007 / LS/CMI risk tools. Serves adult male/female offenders, developmentally delayed and mentally disordered offenders, adolescents, and runs a Chaperone Training Program.
    - **Drew Kingston, PhD** — Chief Clinical Officer (formerly Executive Clinical Director) ZoomInfo and Director of Research; licensed psychologist in California (registered but non-active in Ontario); Senior Scientist at the Royal Ottawa's Institute of Mental Health Research; ATSA Fellow; author of 50+ peer-reviewed works on Static-99R/STABLE-2007 and recidivism. His prominence cuts both ways in litigation (credibility for the defense, but also a heightened standard of care and an authoritative source for the plaintiff's standard-of-care expert).
    - **Katie Palacios** — public records confirm a LinkedIn profile associating her with HOPE Program (linkedin.com/in/katie-palacios-064135186); named defendant as an individual clinician and Site Director. Her public footprint is otherwise thin; her BBS/Board of Psychology license type, disciplinary history, CASOMB certification level, and specific role should be developed through a license lookup and discovery.
- **CPC (Counseling & Psychotherapy Center, Inc. / CPC America)**: multistate (11 states, 80+ locations); LinkedIn uses the proprietary "RULE" program; funded by government contracts, Medicaid, insurance, and self-pay. Not confirmed as a California CDCR SOMP contractor (a Massachusetts-named affiliate appears in national provider listings). CPC is useful primarily as an **industry comparator** for completion criteria, compensation, and program structure rather than as a defendant analogue.

8. Employee/insider perspectives on HOPE

- **Glassdoor**: HOPE Program rated **3.4/5** (25 reviews); 49% would recommend; compensation/benefits rated 2.8/5. Pros cited include "driven staff," Glassdoor training, a 4-day work week, and a strong curriculum/clinical supervision. Recurring cons: "Poor management, micromanagement, overworked and underpaid"; Indeed "The number of clients per clinician is consistently too high which leaves us feeling burnt out and like we are not providing the level of care that we would like to be"; Indeed "Overworked,

underpaid, meat grinder of a job … Turnover is high"; Indeed and references to a "strict authoritarian … micromanaging clinical director."

- **Indeed**: one clinician described caseload as "1-2 groups per day, 9 clients per group, and each client had to be seen once a month for individual sessions." HOPE's official responses state that "client direct contact times are at the industry standards for mental health and align with governing body expectations."
- **Litigation relevance**: high caseloads, high turnover, and clinician burnout support an inference that individualized completion determinations may have been cursory or driven by throughput/retention rather than individualized clinical judgment — relevant to the negligence count and to rebutting a "reasonable clinical judgment" defense. Turnover also ties directly to the CASOMB complaint pattern about clients losing maintenance-phase status when reassigned between clinicians/agencies.

9. Legal precedent & analogous doctrine

- **No California appellate case squarely holds a CASOMB-certified SOT provider liable for fraudulent/negligent completion determinations** was located — this appears to be a matter of first impression, which makes the § 3574 framework and the ACSOL settlement history especially important as the standard-of-conduct baseline.
- **Bane Act (Civ. Code § 52.1)**: reaches private persons "whether or not acting under color of law," FindLaw and requires interference with a right "by threat, intimidation, or coercion." *Cornell v. City & County of San Francisco* (2017) held that the coercion need not be independent of the rights violation and that specific intent — or reckless disregard of the right — suffices. *Shoyoye v. County of L.A.* (2012) bars claims premised on mere negligence or clerical error. Application here is novel: plaintiff would need to identify a protected liberty interest (e.g., freedom from unlawfully prolonged treatment/parole) interfered with by coercive conduct undertaken with specific intent or reckless disregard. The "you cannot complete / you cannot be discharged" leverage documented in the ACSOL litigation is the natural coercion hook, but this is the hardest of the pleaded theories.
- **Mandated-reporter immunity is a red herring but must be addressed**: California's absolute immunity (Pen. Code § 11172; *Krikorian v. Barry*; *Robbins v. Hamburger Home for Girls*) applies to *child-abuse reports*. It does **not** immunize routine treatment-completion/retention determinations or progress reports to parole. There is no analogous absolute immunity for a provider's PC 3008 / § 3574 completion recommendations. Defendants may nonetheless assert **Civil Code § 47(b) litigation/official-proceeding privilege** or common-law quasi-judicial immunity for statements made to parole authorities — a live risk to brief early.

- **UCL (Bus. & Prof. Code § 17200)**: reaches "any unlawful, unfair or fraudulent business act or practice." CA A violation of § 3574(f) (retention without documented good cause / without a signed Form 3043), or of CASOMB certification requirements, supplies the predicate "unlawful" act; retaining clients for revenue past clinical necessity fits the "unfair" prong; misrepresenting completion status to clients or parole fits "fraudulent." UCL remedies are limited to restitution and injunctive relief (no damages), so this count pairs with the fraud/IIED claims for monetary recovery.

Details

The principal primary sources supporting the above are: 15 CCR § 3574 (Cornell LII full text, reflecting the 4/17/2025 amendment); the CDCR rulemaking record NCR 25-01 (documenting the ACSOL settlement basis, the LS/CMI ≥ 11 and Stable-2007 ≥ 4 retention criteria, and Form 3043); the CASOMB 2025 Year-End Report (complaint volumes, carryover complaints on failure to credit prior treatment, sanctions, 69 agencies); the CASOMB Treatment Completion Guidelines (April 2018) (definition of successful completion; "discharge planning starts at intake"; audit authority); the CASOMB Complaint Procedure (rev. 9/2025); ACSOL litigation postings (2023–2024); the HOPE Program website (founding, leadership, locations, "largest" claim); the CDCR FY2019-20 SOMP Outcome Evaluation Report (HOPE confirmed as provider; dosage minimums); and CPC's corporate/LinkedIn materials (funding sources, footprint).

Recommendations

1. **Anchor the liability theory to § 3574(f) and the ACSOL settlement.** Plead that after the one-year CTM, retention required a written, signed, good-cause justification on CDCR Form 3043. In discovery, obtain every 3043 / 1502 / CTM record for the plaintiff and test whether the enumerated good-cause criteria (LS/CMI ≥ 11; Stable-2007 ≥ 4; adverse polygraph; qualifying parole violation) were actually met and documented. Absence or fabrication of that documentation is the factual core of the negligence-per-se and UCL-"unlawful" claims. **Threshold that changes the case:** if the Form 3043s show properly documented good cause tied to the enumerated criteria, the negligence/fraud theory weakens substantially and the case narrows to whether the underlying scores/assessments were themselves manipulated.

2. **Quantify the financial incentive.** Serve a CPRA request on CDCR's Office of Business Services / Contracts Management Branch for the statewide SOT contract (reference C5613216-D and any successors), including the Exhibit B-1 Rate Sheet, and run an Open FI$Cal vendor-payment search for "HOPE Psychotherapy." Establishing per-session or per-client-month billing converts the incentive theory from argument to evidence, strengthening the "unfair" UCL prong and any punitive-damages claim.

3. **Develop the CASOMB/BBS record.** File records requests with CASOMB (any complaints naming HOPE / Palacios / Kingston; any audit of HOPE's completion decision process; sanction history) and BBS / Board of Psychology license lookups for Palacios and Kingston (license type and disciplinary history). Even anonymized CASOMB trend data (top complaint category = failure to credit prior treatment) is admissible context for the industry standard of care.

4. **Retain a CASOMB-certified standard-of-care expert** to opine that (a) discharge planning must begin at intake and completion "should never come as a surprise to the client," and (b) HOPE's conduct deviated from the 2018 Guidelines and § 3574. Use HOPE's own leadership scholarship (Kingston's publications on RNR and dynamic risk) to define the standard of care against which the individual clinician (Palacios) is measured.

5. **Pre-empt the privilege/immunity defenses.** Brief early that Pen. Code § 11172 mandated-reporter immunity is inapplicable, and research whether Civil Code § 47(b) litigation privilege or common-law quasi-judicial immunity could shield reports to parole. Frame the actionable conduct as the *completion determination and retention decision* (a clinical and business act) rather than merely the communication to parole, to sidestep any communication-based privilege.

6. **Use the employee-review evidence as corroboration, not proof.** The high-caseload / high-turnover / "meat grinder" reviews support an inference that completion decisions were not individualized, but they are anonymous and self-selected; corroborate with discovery (personnel files, caseload data, clinician session notes, and internal completion-review policies).

Caveats

- **First-impression posture**: no located California case imposes liability on a CASOMB provider for completion-decision misconduct; the theories rely on statutes and regulations applied by analogy.
- **Regulatory timing is decisive**: § 3574's key good-cause / Form-3043 requirements became fully effective 4/17/2025 (emergency version 10/14/2024). If the conduct predates October 2024, the pre-regulation landscape governs — the unwritten policy the ACSOL suit attacked, plus the 2018 Guidelines and CASOMB certification requirements — a weaker but still viable standard-of-care basis. **Confirm the operative dates of the alleged conduct**, as this dictates which standard applies.
- **Financial specifics unverified**: the per-unit reimbursement rate, the C5613216-D contract value, and any dollar awards to HOPE are not in public sources; the incentive argument is structurally supported but not yet quantified.

- **No named adverse record found** for HOPE / Palacios / Kingston / CPC in CASOMB, BBS, or public court databases via open-web search; absence of a public record is not proof of a clean record — pursue direct records requests.
- **HOPE marketing claims** ("California's largest," high completion / low recidivism) are self-published; the "majority of participants who complete the program avoid reoffending" statement appears in a HOPE blog post that also uses a composite client ("Mark, name changed"). Treat HOPE's outcome claims as unverified marketing, not established fact.
- Sources such as Glassdoor and Indeed are anonymous and self-selected; useful for investigative leads and corroboration, not as standalone proof.